

TWIN SUCCESS

In Janani Express, when a pregnant woman goes into labour, her relatives or anganwadi workers call a helpline to summon an ambulance. The call centre then directs the nearest ambulance to take her to the hospital for safe institutional delivery.



In Janani Suraksha, a pregnant woman, apart from safe delivery, gets Rs 1,400 in cash while her *prerak* (handler), gets Rs 350.



Under Matri Shakti, each pregnant woman would be monitored and a red card issued in case of any complication and her medical history would also be maintained at the hospital.



Currently, the Bhopal-based call centre coordinates ambulance services in its nearby districts, but will soon be upgraded to manage the service in the entire state.



themselves. On any given day, the maternity ward in Vidisha district hospital is almost full. Says 35-year-old Sunita Ahirwar, a Dalit woman from the nearby Khari village, "I had my first delivery at home. But when I heard about this hospital, I decided to come here for my second delivery." Unlike Ahirwar, 21-year-old Preeti from Dyuparia village is the first in her family to come to a hospital for delivery. Impressed with the services, she says, "From now on, every woman in my village would come to a hospital for her delivery."

The scheme relies upon the anganwadi network for its management at the grassroots. It is one of the few instances where two government departments—women and child welfare and public health and family welfare—have come together. The health department has in fact bankrolled the entire hike in honorarium announced for anganwadi workers to enlist their whole-hearted support for the scheme. But the Government needs to overcome the issue of shortage of doctors. For despite its three attempts over the last two years to recruit more doctors, there have been few takers for jobs in rural areas.

Buoyed by the success of these schemes, the state Government plans to launch another ambitious programme Matri Shakti, which is currently running as a pilot project in eight blocks. Under the proposed plan, each pregnant woman would be monitored through an anganwadi network for medical history of miscarriages, anaemia, weight problems, etc. She would then be issued a red card and her medical history maintained at the hospital where she would eventually deliver. Says 24-year-old Sarwari from village Pipaliya, "This card means some complications in my pregnancy. Therefore, I need to go for regular check-ups and get my delivery done at the hospital."

For the time being though, Madhya Pradesh has emerged top of the group even in Centre's ratings, scoring 12 out of 14 on a scale for NRHM schemes, bagging the top slot among the nine poor performing states in the country in the fourth successive nation-wide evaluation. This is perhaps the state's first step towards erasing the BIMARU tag. ■

HAPPY AND LUCKY: A woman who had a safe delivery at a government hospital

but will soon be upgraded to manage the services in the entire state.

Dr Ajay Goenka of Chirayu Hospital in Bhopal, one of the few super specialty private medical centres in the state, says, "These twin schemes have driven many private maternity and nursing homes out of business." In fact, many nursing homes are now lining up to be enlisted as a private institution for public health and become part of this government-backed health scheme. While figures for private nursing homes are not available, the number of deliveries being performed at the government hospitals serves as a useful pointer. The number of institutional deliveries in J.P. Hospital in Bhopal alone has seen a threefold rise, from 1,500 in 2007 to 4,400 in 2008. Beyond the clutter of official figures, the success of the schemes is borne out by the experiences of beneficiaries